

ONTARIO
SUPERIOR COURT OF JUSTICE

BETWEEN:

DOUGLAS WEST

Plaintiff

– and –

ALEX J. HANNA and TRANSFORCE
ADMINISTRATION INC. aka
TRANSFORCE ADMINISTRATION
INC./ADMINISTRATION TRANSFORCE
INC., CANPAR TRANSPORT
L.P./TRANSPORT CANPAR, S.E.C. and
TF1 TRANSPORT 2 L.P./TRANSPORT
TF1 2, S.E.C.

Defendants

Ian S. Greenway, for the Plaintiff

Michael Blinick and Deborah Ikede, for the
Defendants

HEARD: January 7, 8, 9, 10, 13, 14, 15, 17,
20, 21 and March 21, 2025

REASONS FOR JUDGMENT

S.E. FRASER, J.:

TABLE OF CONTENTS

I.	Overview	3
II.	Issues	4
III.	Analysis of Evidence	4
A.	Credibility and Reliability	5
B.	Documentary Evidence	5
C.	Lay Evidence	5

(a)	Douglas West	5
(i)	The Accident	9
(ii)	Injuries	10
(iii)	February 26, 2017 Accident.....	10
(iv)	Police Incident	11
(v)	Admission to Algonquin College.....	11
(vi)	Life After the Accident	12
(vii)	Present Condition.....	13
(viii)	Credibility	14
(b)	Evidence of Sherry West	15
(c)	Evidence of Jillian Robinson	16
D.	Medical, Psychological and Other Opinion Evidence	17
(a)	Dr. Majinder Randhawa (Family Physician)	17
(b)	Dr. Kristopher Edwards (Chiropractor)	18
(c)	Dr. Felix Thomas (Psychologist)	19
(d)	Christopher Agaton (Rule 53 Expert)	20
(e)	Dr. Arnold Miners (Rule 53 Expert).....	21
(f)	Dr. Darryl Appleton (Rule 53 Psychiatrist)	22
(g)	Dr. Gordon Ko (Rule 53 Expert Psychiatry)	24
(h)	Marla Tennen (Rule 53 Expert)	25
(i)	Saqib Durrani (Rule 53 Expert)	25
(j)	Dr. Michael Boucher (Rule 53 Pain Specialist).....	25
(k)	Dr. Andrew Holland (Rule 53 Life Care Planner).....	27
(l)	Dr. Allan Peterkin (Rule 53 Psychiatrist)	27
(m)	Dr. Freedman (Neuropsychologist)	29
IV.	Legal Analysis	30
A.	Causation.....	31
B.	Findings.....	32
(a)	Pre-Accident Health.....	32
(b)	Accident	34
(c)	Injuries Sustained.....	35
C.	Did the Accident Cause the Injuries?.....	36
(d)	Mitigation.....	37
D.	Quantifying Damages	38

(a)	General Damages	38
(i)	Positions of the Parties.....	38
(ii)	Analysis.....	38
(iii)	Conclusion on Non-Pecuniary Losses	39
(b)	Past Economic Loss	39
(i)	Positions of the Parties.....	39
(ii)	Evidence.....	39
(iii)	Conclusion on Past Income Losses.....	42
(c)	Future Economic Loss	42
(i)	Test.....	42
(ii)	Evidence.....	42
(iii)	Conclusion on Future Economic Loss	43
(d)	Future Cost of Care	43
(e)	Out of Pocket Expenses	45
E.	Threshold Governing Principles	45
F.	Do the Injuries Meet the Threshold?	48
V.	Conclusion	48

I. Overview

- [1] Douglas West sues for damages after being rear ended by a cube van on January 16, 2017, driven by the Defendant Alex Hanna.
- [2] Liability in this case is admitted. Damages are in dispute.
- [3] It is also an issue for trial whether Mr. West meets the threshold for compensation under the *Insurance Act*, R.S.O. 1990, c. I.8, as amended. The Defendants assert that Mr. West did not suffer a permanent, serious impairment of an important physical, mental or psychological function because of the collision.
- [4] They argue that the injuries suffered from this accident were soft tissue injuries only, that other events were more significant, and that Mr. West was unlikely to ever sustain employment.
- [5] Mr. West did not have a perfect life before the accident, but he was getting by. He had a romantic partner. He stood in the shoes of a parent to her young child. He saw friends and family and he was working towards a new career in radiation safety. The first step was getting his high school diploma.

- [6] He had had difficulty in school, and challenges growing up. I learned about him from his mom, who has her own memory challenges because of a brain injury, his former romantic partner, and his family doctor who treated him both before and after the accident.
- [7] Mr. West had an uneven work history. Before the accident, he had not worked regularly since 2015. Between 2007 and 2014, he had lived in Western Canada, working in the oil fields off and on, and in the kitchens of restaurants like Boston Pizza.
- [8] He used street drugs and, on at least three occasions, went to the hospital in crisis. He was diagnosed with an adjustment disorder and a bipolar disorder prior to the accident. The experts before me disagreed on whether the later diagnosis was the correct one, given that he was using drugs when it was made.
- [9] Mr. West is no longer getting by. His life changed drastically in the months following the accident and he is now a recluse. He smokes a lot of marijuana for pain. He is angry and upset about how the accident changed his life and says he died on that day.
- [10] The accident was also not the last unfortunate thing to happen to Mr. West. On February 26, 2017, Mr. West was driving a car when it went into a ditch. On May 9, 2017, he had an altercation with police.

II. Issues

- [11] The issues that I must decide are:
 - a. Has Mr. West proven on a balance of probabilities that the Defendants' breach of the standard of care, alone or in combination with other factors, was a necessary factor in bringing about the harm to Mr. West?
 - b. If so, what is the appropriate damage award to restore the Plaintiff?
 - c. Does the Plaintiff fall within exception of s. 267.5 of the *Insurance Act*, R.S.O. 1990, c. I.8, to entitle him to damages? In other words, has Mr. West sustained a permanent, serious impairment of an important physical, mental or psychological function because of the collision?
- [12] Both the threshold question and the causation questions require me to consider the evidence and make findings about what happened. I must determine what Mr. West's life would have looked like, but for the accident.

III. Analysis of Evidence

- [13] I begin with the principles of credibility and reliability, then assess the lay evidence, starting with the Plaintiff. Then I turn to the evidence of the participant and Rule 53 experts.

A. Credibility and Reliability

- [14] Credibility is about the honesty or veracity of a witness' testimony. Reliability has to do with the accuracy of a witness' testimony which is often assessed by the witness's ability to observe, recall and recount the events at issue: *R. v. Sanichar*, 2012 ONCA 117, at paras. 69 – 70 per Laskin J.A, as adopted by the Supreme Court of Canada (2013 SCC 4).
- [15] As set out in *R. v. Kruk*, 2024 SCC 7, 489 D.L.R. (4th) 385, at para. 81, when assessing credibility and reliability of the evidence of witnesses, I must remain grounded in the totality of the evidence. At the same time, I must evaluate the testimony of each witness and make determinations that are entirely personal and particular to that individual.
- [16] I can accept some, none or all of a witness's evidence.
- [17] The credibility and reliability of Mr. West is central to the case. This is because clinicians based their findings on his reports to them. Much of what he reported is not, by the nature of the injury, able to be objectively observed.

B. Documentary Evidence

- [18] The parties filed a joint document brief. The parties agreed that the documents were authentic and admissible for the truth of their contents. The parties agree that where a record contains a statement, the statement is admissible for the fact that it was said, but not for the truth of its contents.
- [19] I note that the health records tendered detail that Mr. West presented with psychosis at the Lakeridge Hospital on August 18, 2014 and was discharged on August 21, 2014 with a diagnosis of psychosis. There, Mr. West was under the care of a psychiatrist Dr. Racicovschi.
- [20] In June, 2016, he attended Lakeridge Health reporting that he had a rash all over his face and some on his chest and right arm. He reported that this started after he had been taking cocaine and so he stopped.
- [21] In the Fall of 2016, Mr. West again attended hospital in crisis.

C. Lay Evidence

(a) Douglas West

- [22] Douglas (Doug) West was born in 1985 and at the time of the trial was 39 years old, now 40. He was born in Scarborough and grew up in Oshawa and was raised by his mother Sherry West. Close family members included his mother, his brother, his stepfather and his grandfather. He did not mention his birth father. His first job was as a paper boy.
- [23] He testified with some difficulty. He struggled to remember details. At times he was frustrated and combative. At other times, he was overwhelmed by the process. He accused

Defence counsel as terrorizing him when he asked questions in a respectful fashion. He was unable to tolerate the process from time to time.

- [24] Mr. West stayed in school until grade 11 and then worked for a moving company with a crew of two. He describes lifting objects of 500 pounds alone and objects like a piano, which he estimated to be 4,000 pounds, with another person using a skid and pushing and pulling. He first said that he worked 40 hours per week and then corrected himself to one to three times a week.
- [25] He next worked at Cathy's Café as a cook when he was 16.
- [26] Mr. West described moving to Western Canada with a friend when he was 22. That would put the date in 2007. He worked at Ackland Grainger in a warehouse job on an oil field. He went home every night. He did that for about either three or six months before working in the oil field. He worked as a floor hand on drilling rigs which involved living in a camp. This job entailed heavy lifting making connections with the pipe which drills in the ground and cleaning out mud to prevent a blowout. In short, heavy lifting (about 100 pounds) and cleaning all day. The schedule was about 20 days on and 10 days off. He left when the oil patch was booming but recalls that when he arrived in Alberta that 300,000 people had just been laid off.
- [27] His memory was fuzzy, and testimony disjointed about his time out West. He had difficulty placing events in a proper chronology.
- [28] For a while in Alberta, he was living with his ex-fiancé Deanna. He said that he was not saving much money because he was only getting 14 days on, and then his family started dying so he moved back home. He also said that Deanna was living in Calgary and they broke up, so he moved to Victoria.
- [29] He worked for Calibre Drilling working 14 days on and 7 off as a floor hand. He moved up to motor hand (where you take care of the motors), but that did not last as they were laying off a lot of people. As a floor hand, you must carry mud which is in 50-pound bags, and he would have to pack 1,000 of them. He describes the work as very hard.
- [30] After Calibre Drilling, he worked as a line cook at Boston Pizza in Edmonton. He thinks he was working about 50 hours. He then moved to Victoria and worked in kitchens. He was a supervisor at Boston Pizza for a time. He would have to be on his feet eight hours a day. He thinks he lived in Victoria somewhere between 14 months, 16 months, 18 months and two years (he gave all these answers at different times). At one point, he lived in Vancouver.
- [31] After Victoria, he returned to Edmonton and worked for Predator Drilling. He lived with his uncle. He thinks he was 26 or 27 when he moved back. His uncle died and his aunt also died.
- [32] When living in Vancouver, he worked at the Donnelly Group in Vancouver for a 20-day stint as a floor hand in Alberta. He worked at Predator Drilling for about a year.

- [33] He worked for Trinidad Drilling in 2014. He moved back to Ontario in 2015. He did not earn income in 2014.
- [34] He then moved back to Ontario. He described his mother being sick and his aunt and uncle dying. He wanted to come home for what he thought would be three funerals. He worked at Boston Pizza for about two weeks when he returned.
- [35] Records show that on August 18, 2014, he was the subject of a mental health assessment at Lakeridge Hospital. Dr. Racicovschi's discharge note states that he was acutely psychotic when he arrived and was having delusions. He was placed on a Form 1 under the *Mental Health Act*. During the short admission he improved. Dr. Racicovschi noted that this "might give us an idea that the cause of his psychosis is marijuana abuse or other drugs or that the patient was in withdrawal". He was discharged with a diagnosis of Psychosis NOS.
- [36] Asked about this admission, Mr. West stated that he experienced homelessness at the time.
- [37] In January, 2015, Dr. Randhawa, his family doctor, diagnosed him with an adjustment disorder. Mr. West testified that he was not informed of the diagnosis.
- [38] In February 2015, Dr. Randhawa noted increased anxiety and repeated prescriptions for Imovane and Clonazepam. Mr. West described this as a time of having to sort things through but had no specific recollection about this time. He acknowledges that he had difficulty sleeping at this time and some anxiety. He also acknowledged that Dr. Randhawa discussed the risk of addiction with him.
- [39] Also in February 2015, Mr. West attended at the hospital because he got in a fight in a bar and got bitten by someone.
- [40] Eventually, Mr. West decided to work towards a second career in Ontario as a Radiation Safety Officer after hearing that Ontario Power Generation had hired 75 persons from Australia to do that job. To do so, he needed to complete high school, so he signed up at an adult high school. He needed physics, math and English and he needed 65% for him to pass to get into Algonquin College for the Radiation Safety Officer Program.
- [41] His evidence about what he took in school was not reliable. However, it appears that he returned to school in early 2016.
- [42] His Durham Continuing Education Report Card dated June 2016 shows the following courses, completion dates and final marks:

Math 3C1, June 22, 2016, 51%

English 4C1, June 22, 2016, 65%

Chemistry 4C1 to be completed August 15, 2016

Math 4C1 to be completed July 28, 2016

- [43] Mr. West told the Court that he had to stop taking the chemistry course because of cysts on his face. The medical records tendered show that he was seen by Dr. Lee for an abscess cheek on August 18, 2016, and then at the Port Perry Hospital in September 2016.
- [44] Records show that he transferred to the Oshawa Campus of Archbishop Anthony Meagher on September 7, 2016 and was enrolled until October 28, 2016. However, the school records show that he stopped attending as of September 19, 2016. He was then registered at the Ajax Campus between March 23, 2017, and May 30, 2017 and during that time he completed one credit in physics and earned two equivalent credits (mature credits). When he started, the guidance counsellor told him that it would be a hard experience, and he found it to be so.
- [45] Around the time he was pursuing adult education, Mr. West became involved with Jillian (Jill) Robinson who he met through a friend from high school. She was a hard-working woman with a family scrap yard business. He wanted a career, having been tired of being laid off. In the beginning, he had his own apartment, and she had her own place and a daughter. Even though he had his own apartment, he stayed with Jill often.
- [46] He was asked about the period before the accident. His family doctor, Dr. Randhawa, had diagnosed him with anxiety and an adjustment disorder. Mr. West did not recall being told that he had an adjustment disorder. He testified that in the year before the accident that some of his family members were sick, he was going to school, he had a girlfriend who had a child and that as a result he was under a lot of stress.
- [47] He met with a psychiatrist, Dr. Racicovschi, in 2016 because of the stress and anxiety. He was prescribed Clonazepam for the anxiety.
- [48] Mr. West had a difficult time describing his life in the time just prior to the accident. He was able to tell the Court that he was going to school and living with Jillian and her child. He had played hockey and gone to the gym in the past.
- [49] On October 7, 2016, Dr. Racicovschi wrote that she had seen Mr. West in the outpatient clinic for depression and anxiety. He complained of a mood 3/10 and decreased sleep. He was smoking 1 g of cannabis a day and had a history of cocaine use but reported to her that he had not used in the past six months. She summarized her opinion as follows:

In summary, Douglas West is a 31-year-old man with bipolar spectrum disorder, generalized anxiety disorder and is currently in a depressive phase. He also has a history of substance abuse. We also need to rule out attention deficit hyperactivity disorder. We have thus given him a test for ADHD to complete and return back in follow-up. Douglas has also been educated about cannabis rebound anxiety and depression and we have advised him to make a serious attempt at stopping cannabis usage. We also warned him of the dangers of benzodiazepine addiction.

- [50] Mr. West stated that he did not take 2.5 mg Clonazepam per day as recorded in her note and that he was not smoking that much cannabis. In November 2016, he received a prescription for Percocet which he stated that he took for the cysts.
- [51] Despite having reported that he had not used cocaine in six months, it appears that he used cocaine in June, 2016 because he attended at the hospital on June 29, 2016 with a face rash after using bad cocaine.
- [52] In December 2016, he again attended at the hospital and was given medication which was over the counter medication. The clinical note states that he threw the medication on the bed and told staff there that he did not need “that stuff” and left without the medication. It appears that he was looking for Percocet.
- [53] He stated that once he was hospitalized for a mental health reason before the accident because he did not have a place to live.
- [54] He stated that he was focussed on his career and described the necessary prerequisites for the Radiation Safety Officer program. He needed an English credit at 65%, 60% in math and a physics or chemistry credit. He was doing the physics course by correspondence. He described being on a first name basis with the admissions officer at Algonquin College.
- [55] As of the date of the accident, he had completed math and English and he still needed to complete physics.
- [56] While his girlfriend at the time Ms. Robinson described him as being a daily cannabis smoker, Mr. West denies that he smoked this quantity before the accident.

(i) The Accident

- [57] Just before the accident, Mr. West had been in the guidance counsellor’s office where he was doing his upgrading, speaking with an admissions person at Algonquin College. He recalls the pamphlet being in the car.
- [58] Mr. West described being a belted passenger and being hit from behind by a Canpar cube van loaded with cargo. His car then hit the car in front of him which had been five feet ahead. He remembered saying to himself three times “I am not going to die” and he came to. He described that when he said that he came to, it was from a vision of blackness.
- [59] He said airbags deployed. The rear window shattered. He blacked out. “You get hit. I guess and then you really black out. You put your head, like your head well, I didn’t put my head down. I lifted my head off and I think that’s where the confusion is here.” He stated that his hair was on the steering wheel, and he thinks he hurt his shoulder on the steering wheel.
- [60] He was in the vehicle for about 15 seconds before he could exit it. The other driver came to see if he was okay, but he couldn’t really talk – he was screaming. Jill came and took him to the hospital. He did not want to be taken from the scene by paramedics because he was driving a rental and he had to deal with the rental. He described his mind being

scattered. He thought he was going to die. There was an impact with the headrest airbag that he was unable to describe fully.

(ii) Injuries

- [61] Mr. West described his injuries as a bruised ribcage (left side), bruised shoulder (left side), sore head, and sore back. He also mentioned that he couldn't really feel his pinky toe.
- [62] While some medical reports note Mr. West as saying that there was no loss of consciousness, Mr. West testified that he lost consciousness.
- [63] He was taken to the Port Perry Hospital by Ms. Robinson and clinicians determined that his neck was not broken. Hospital records show that he reported being hit by a cube van and that since then he has had mid-back pain and right neck pain and mild pain in his left shoulder. The diagnosis was cervical/thoracic strain.
- [64] On January 16, 2017, Mr. West went to his family doctor's clinic and saw Dr. Lee. He reported that the headrest airbag hit the back of his head and that he was confused for one hour after the accident. His complaints were continuous headache, continuous back pain, nausea, left shoulder pain, fatigue and dizziness. He was prescribed Gabapentin and was encouraged to get physiotherapy. He looked for it, but he did not know how he was going to get it until he found out that OHIP would cover the physiotherapy. He did not start it until mid-April. He described being a student and having no money. The records show that he began physiotherapy on April 16, 2017. He also said that his insurance denied him physiotherapy.
- [65] For the month following the accident, he said that he could not think straight. He was delusional. He had weird flashes like headaches on the right side. He did not know what was going on and wanted to finish his schooling. He felt a lot of anger. The injuries did not get better, he said they got worse.
- [66] In March 2017, he tried to return to school at "Archbishop" in a class on a computer. He said that the attendance was not good and that this was in part due to medical appointments. He managed to obtain his credit. I find that he persevered through pain to do so.

(iii) February 26, 2017 Accident

- [67] In February 2017, Mr. West was driving when he went off the road at 30/40 km/hr into a ditch. He described the accident as minor. There was some damage to the front end of the car. The side air bag deployed. No paramedics attended. He did not go to the hospital. He has some minor knee injury. The police charged him with impaired driving. He states that this is because of the medication, Gabapentin and Oxycontin, that he was taking at the time. Oxycontin was a medication that he got off the street because he was looking for relief from the pain. He also said that it was the long drive that got him. His license was suspended for three months at the time and later, after a trial, for five years.

(iv) Police Incident

[68] Some time after the second accident, Mr. West was involved in an altercation with police. He could not recall the details other than that he and Jillian got into an argument, and he had to leave the house.

[69] However, Dr. Shreeram Prasad of the Toronto Western Hospital wrote in a clinical note of December 14, 2018, that Mr. West reported:

He had another accident on July 1, 2017 when his car ran into a pole. He did not sustain any injuries. He was drunk at that time. He was caught by police who “jumped on his neck.” He reports that he had an MRI of his brain which is said to be normal. He says that all these accidents added on to his neck and shoulder problems.

[70] Mr. West says that this report is totally inaccurate. Police did not jump on his neck. He did not hit a pole. I have been provided with no other evidence of a July 1, 2017 accident.

[71] Health records indicate that he attended at the Bowmanville Hospital on June 1, 2017 with a toxic intake of Tylenol, Advil and Aspirin and he was trying to slow down on his medications from the original incident.

[72] Dr. Daniel Cohen assessed Mr. West on September 19, 2017, and noted:

Mr. West indicated that on May 9, 2017 he was travelling in a taxi and had passed out in the rear of the vehicle because of sleep medication. He noted that the taxi driver drove him to the police headquarters and that he was subsequently aggressively removed from the vehicle and assaulted by a police officer who stomped on his chest. He said that he was charged with failure to leave the premises and being drunk in public. He acknowledged contesting these charges as well. He said that he believes he suffered fractured ribs in this incident.

[73] The details and date of the interaction with police are shaky.

(v) Admission to Algonquin College

[74] Mr. West stated that he was admitted to the Radiation Safety Officer Program in Pembroke, Ontario but that he could not attend because of his post-accident condition. Two records were produced. First is a letter dated May 24, 2016, from Algonquin College which documents receiving Mr. West’s application. It required confirmation of completion of the required math and science courses.

[75] The second is letter dated June 23, 2016, advising Mr. West that he had been placed on a conditional waiting list. His acceptance would depend on there being a cancellation in the program.

- [76] There is no record of Mr. West providing Algonquin College with his transcripts in 2017 after completion of his math and physics courses. There is also no letter of acceptance. I find that he was not admitted but conditionally waitlisted.

(vi) Life After the Accident

- [77] Mr. West described how after the accident he was in constant pain and that it has been that way for the past eight years. Immediately after the accident, Jill was working full-time. He would wake up and then go back to bed about two hours later. He would take pills to alleviate the pain. He would then sleep on and off in a permanent daze, just laying there to pass the time. He felt like dying.
- [78] He said that this has been very stressful. He has lost everything. Jill's daughter told him that she did not like him anymore more because he just lay on the couch. She said he was lazy and that she wanted her father back.
- [79] He was sad at times. Angry at other times. He had a difficult time tolerating the courtroom environment.
- [80] Asked in cross-examination about the mental health challenges that he faced before the accident, Mr. West replied not to the extent that he did after it, noting:
- I forget my name, I forgot my birthday, I lost my girlfriend, I lost my child, I lost my mother, I used to work full-time for a moving company. Worked for an oil rig company. No, - now you (*sic*) can't even lift my laundry basket.
- [81] Asked about the anxiety before the accident he connected it to family members dying and having difficulty sleeping and that this was not the same as dying in a car accident.
- [82] Mr. West appeared to continue with school taking a physics course by correspondence, noted in the school records as hybrid online. He passed that course with a grade of 66% as noted in the Final Report Card dated May 20, 2017.
- [83] He told the Court that when he goes shopping, he cannot move for three days after.
- [84] He has consumed 10 grams of marijuana a day since the accident and said that he does so because he is angry. He last used cocaine about six months before testifying.
- [85] His shoulder and neck continue to provide him with 24 hour a day pain. To turn his head, he must turn his shoulders and then his back starts hurting.
- [86] He has lived on his own in a bachelor apartment for four years. He received \$110,000.08 by way of an accident benefit settlement and he used that settlement to pay off his debts.
- [87] He liked seeing Dr. Thomas, but he could not afford his services after he ran out of money. He has an outstanding bill for services.

- [88] Nerve blocks were recommended to him. He received one on June 1, 2018 but he reported that it was very painful, that the needle pain lasted six days and that he could not tolerate it. He preferred to have someone to talk to.
- [89] On March 6, 2019, Mr. West was admitted involuntarily to the mental health unit of the Lakeridge Hospital under a Form 3 and discharged March 9, 2019 with a referral to Pinewood for Addiction Services. It was noted that he had been placed on Olanzapine 25 mg, much higher than the 5 mg dose prescribed in 2016 and Epival. It was noted that decreasing his cocaine use would benefit his mental health. Mr. West admits to struggling with cocaine use in 2019.
- [90] Since 2017, he has received no care from a social worker other than his brain injury worker.

(vii) *Present Condition*

- [91] Mr. West does not like to rely on other people. He appeared to reflect genuinely on what he perceived as being a burden to others that he did not like. He appreciated that others had to take time off work to get him to appointments. He presently lives on his own and cooks using the microwave. He does not go out. He gets food from the food bank, whatever they can give him. It hurts to shop. He no longer drives having lost his license. He spends \$2,000 a year on cannabis which he purchases on reserve as it is cheaper. Receipts for the purchase of cannabis from the Peace Pipe, the Old West Cannabis Company and Tokyo Smoke were tendered. Mr. West described this as medicine.
- [92] Mr. West has the assistance of a brain injury worker from the Brain Injury Association of Durham. He said that he helps him a lot. It is not clear how frequently he sees him and Mr. West's responses were once a month and once a week and then whenever I can. Mr. West testified that he helps him with all the things he needs to do including taxes.
- [93] When asked about how he became connected to the worker, Mr. West advised that he had been in the mental health facility and then spent time at Pinewood for rehabilitation.
- [94] His mom suffered a brain injury, and he finds it hard to talk to her as well. She reports, as later described, that he often does not wish to see her. He does not speak with her or Jill anymore. In short, he is living in poverty, not working, without much health or other social support.
- [95] Mr. West testified that before the accident, his anxiety and depression were minor and that it was really nothing. He compared his life at trial, where he just sits in his apartment in pain to his life pre-accident when he was trying to recalibrate his career and get to college. This included trying to get a pardon, but he did not pursue it after the accident. The Brain Injury Association has classes but he states that he is not able to attend them.
- [96] He would like to have a psychologist because he doesn't have anyone to talk to anymore.
- [97] Mr. West was challenged about delaying treatment for physiotherapy, when the treatment he received was OHIP funded. Mr. West stated that this was during the time that his

girlfriend was going to work, and she would have had to miss work to take him. He did not have a car to get there himself.

- [98] Mr. West received psychotherapy, physiotherapy and other services. He did not receive assistance with personal care or social work or help in his home.

(viii) *Credibility*

- [99] The Defendants argue that Mr. West's evidence was disjointed, contradictory and often temporally inaccurate. They assert that he minimized the challenges that he faced before the accident including that he suffered only minor anxiety before the accident, that he did not rely on Clonazepam to manage his anxiety, that he would be getting a pardon to get a job at Ontario Power Generation, and that he had been accepted to Algonquin College.
- [100] Indeed, there are many challenges with his evidence. He is clearly unable to recount events in the past. I find he tended to minimize the challenges that he faced prior to his accident.
- [101] The Defendants suggest that I should not accept his evidence that he was in school before the accident. However, I accept that he was in school. There were breaks between his courses. Mr. West commenced school in 2016 and was enrolled off and on with breaks into 2017. Mr. West demonstrated to me that his focus in 2016 and the first half of 2017 was to earn credits to get into the Radiation Safety Officer Program.
- [102] I am not concerned that he failed to disclose to post-accident assessors that he was prescribed Imovane and Clonazepam prior to the accident because he disclosed that he had depression and anxiety. If the diagnosis of bipolar disorder was firm, which I will discuss in more detail below, I find both that that did not appear to resonate with Mr. West and that he did not view it as his primary problem.
- [103] I find having observed Mr. West testify, that he suffers from some memory problems and confusion. It is also clear that he has difficulty discussing some of his problems and putting them in chronological order. I am careful about finding fault with his ability to recount things in a linear fashion because it appears to arise from some of the problems that he is experiencing.
- [104] I accept that he minimizes the challenges that he would face to successfully return to school and the difficulties he would face in reaching the goal of becoming a Radiation Safety Officer.
- [105] I find that his minimization of the challenges stems in part because they seem small to him in comparison to the pain that he experienced post-accident and the impact upon his life in all facets. The accident overwhelmed his pre-accident challenges in his mind.
- [106] Despite minimizing pre-accident challenges, I accept his evidence that post-accident he has problems with memory and pain. I accept that he has difficulty lifting and that he experiences confusion. I also accept his evidence that the second accident and the altercation with the police did not cause these problems.

- [107] There were times when humour and affability broke through and you could see endearing qualities spoken about by his mother and Ms. Robinson. That is in stark contrast to the anger about the accident, his present situation, and the losses that he has suffered.

(b) Evidence of Sherry West

- [108] Sherry West is Mr. West's mother. In 2020, she was in an accident and placed in a coma for two weeks. She has suffered a brain injury. She states that her long-term memory is better than her short-term memory. She tired easily in her evidence. Consequently, it was broken up over a few days.
- [109] She has some difficulty remembering details. She described her son as a person completely changed by the accident.
- [110] In his youth and early adulthood, Mr. West was very close to his maternal grandfather. He was also close with her brother and sister and his brother Richard, his brother by his stepfather.
- [111] Mr. West and his grandfather would spend time, including at the cottage together. Mr. West was capable doing all the jobs at the cottage like building docks, painting the cottage and also water skiing, canoeing, and fishing. Mr. West's grandfather passed away in 2015. The cottage was sold at that time. Mr. West helped get the cottage ready for sale by painting the inside of the cottage.
- [112] Mr. West cooked.
- [113] As a child and youth, Mr. West played hockey, he boxed and went trucking (off-roading). She showed us many pictures of Mr. West when he was well and with his family.
- [114] She described his time in the oil fields as going for months at a time and coming back. He had worked for a moving company and on oil rigs. She thought that he lived with her brother between 2013 and 2016.
- [115] She was in Florida at the time of the accident. She came home and saw him two days later. Ms. Robinson and Ms. West did not get along and she was not allowed into the apartment. She recalls speaking with him at the doorway to the apartment and Mr. West had an icepack on his neck and heat pack on his back and told her that he had to go lie down.
- [116] She would see him once a week to bring him Tylenol and heat wraps and try to make sure that he was taking his medication. He would phone her in the middle of the night crying.
- [117] She had her accident June 17, 2020. He was concerned about seeing her because of COVID-19 and they would speak to each other by phone, each lying on the couch. However, then he changed. She reported that he doesn't trust anyone, and he will not come to her house anymore. He is confused all the time. She described having taken him to the hospital for a mental health evaluation but that the doctor let him go.

[118] About eight months prior to the trial, she was at his apartment. All the blinds were closed, and it was like he was in a cave. There was garbage and recycling everywhere. Before the accident, he was spotless when he lived with her.

[119] She testified that the landlord keeps in touch with her. Other tenants report that he howls and screams and has nightmares.

(c) Evidence of Jillian Robinson

[120] Jillian Robinson testified that she met Mr. West in about May or June, 2014 and they soon began a relationship.

[121] She said that when they first met, he was quiet until you got to know him. He was funny and more outgoing as you got to know him. He was well kept together. He was warm and welcoming. He got haircuts every two weeks, went to the gym at the Oshawa Civic Centre and liked swimming. He would walk her two dogs with her. He looked after his appearance.

[122] Mr. West was not working when they first met. He had just come back from Alberta and really wanted to become a Radiation Safety Officer. He smoked cannabis when they met and she described him as smoking a couple in the day.

[123] Prior to the accident he would be in school most mornings and afternoons and then he would go to the gym, workout and swim, and then they would do dinner. He would do dishes, cook dinner or do the laundry and the other things that needed to be done to look after the house. He did quite a bit of outdoor work as well.

[124] Ms. Robinson testified that her daughter adored Mr. West and that he was playful with her. Mr. West had his own apartment.

[125] She was not aware of his break from school in the Fall of 2016. She also was unaware of any cocaine use, or medication for anxiety and depression. She doesn't recall him going to hospital after being bitten in 2015.

[126] At the time of the accident, they were not living together. They started living together in 2017.

[127] She did not believe that he had any emotional or mental health issues leading up to the accident. She was not aware of any medical issues.

[128] She noted at the scene of the accident that the back window was smashed out. He was distraught but she could not remember his physical condition.

[129] After the accident, he stayed with her every night and then moved in after six months. He did not assist with the move.

- [130] She described that his whole personality changed after the accident. He was not communicating; he was distant and closed off with her. It led to the end of their relationship.
- [131] He would lie in the same spot every day. She stated, “he became part of the couch”. He no longer went to the gym. He did not play or engage with her daughter. He closed himself off socially. He did not want to speak with anyone. He would cry a lot of the time and complain about his head and neck. He no longer did any of the housework.
- [132] Mr. West moved out of her house in 2019. She slowly stopped going to visit him. The condition of his apartment was very upsetting. She had to clean his dishes. His garbage was overflowing. He looked scruffy. He would go for months without cutting his hair and it was greasy. The health complaints continued to be the same and focussed on head and neck.
- [133] Asked about the other car accident, she described it as being minor and that he was not overly shaken up about it.
- [134] She would bring groceries for him. He would not carry them. He would come with a cart, and she would carry the rest.
- [135] With respect to his career goals, she stated that he had done a tour of the colleges and that he had applied to one and that they told him what he needed to do to get in but after the accident, everything went downhill and eventually everything stopped. He felt like a failure as all his other friends were doing things and had careers.

D. Medical, Psychological and Other Opinion Evidence

- [136] I now outline the medical and psychological evidence. The parties consented to the written reports of the practitioners being received into evidence as well as their oral testimony.
- (a) Dr. Majinder Randhawa (Family Physician)
- [137] Dr. Majinder Randhawa is a family physician and treated Mr. West for over 25 years and testified as a participant expert. Prior to the accident, he diagnosed him with anxiety and depression as well as an adjustment disorder. An adjustment disorder is depression related to stress and his ability to cope. He referred him to a psychiatrist. He does not think that Mr. West met with a psychiatrist.
- [138] Dr. Randhawa acknowledged that in 2015, Mr. West was having trouble coping with other people. Mr. West did not have coverage to seek private counselling. He was reluctant to take medication.
- [139] He describes Mr. West as being a pleasant patient prior to this accident and that after the accident, he tended to present as agitated or angry. This led to some specialists saying that they could not see him because of his anger.

- [140] Dr. Randhawa did not see him immediately after the accident. He saw him on the next visit and noticed a decreased range of motion in his neck and a muscle spasm with pain radiating with both hands. Mr. West wanted something to alleviate the pain. Dr. Randhawa opined that it was a whiplash injury and referred him to Dr. Ko. In appointments after, Mr. West reported headaches and dizziness.
- [141] Dr. Randhawa noted that he would still come to the practice. The last time that he saw him, he had lost a lot of weight, he was dejected and dishevelled. He noted that he looked older than his actual age.
- [142] After the accident, Mr. West missed appointments, and they had to discharge him from the practice because of it. This did not occur prior to the accident.
- [143] He diagnosed Mr. West with whiplash, post-concussive symptoms, depression, and anxiety.
- [144] I do not agree with the Defendants' submission that his evidence is of limited value. The Defendants point to several challenges with his evidence including his reliance on Mr. West's self-report, his lack of knowledge about Mr. West's employment history, his lack of knowledge of his drug use and that he initially diagnosed Mr. West with whiplash type injuries and diagnosed Mr. West with post-concussive symptoms despite Mr. West not undergoing any diagnostic testing or reporting any symptoms contemporaneously.
- [145] I accept his response that he based his diagnosis on notes from Dr. Ko and historical information. In my view, he was entitled to rely on the specialist's opinion.
- [146] I also do not think it is fair to rely on his assessments for the CPAC machine and ODSP as being the only evidence on behavioural changes he witnessed. Those assessments alone do not reflect the whole of his evidence. Dr. Randhawa witnessed behavioural changes in Mr. West and those changes led to him having difficulty accessing care. I find his evidence of value and persuasive on this issue.

(b) Dr. Kristopher Edwards (Chiropractor)

- [147] Dr. Kristopher Edwards is a chiropractor licensed to practice in the Province of Ontario. He treated Mr. West for chronic pain and concussion symptoms and noted that the delay in seeking treatment was likely contributing to the chronicity of his issues.
- [148] Mr. West first met him on September 7, 2017 after having been diagnosed with a concussion. He describes that at the beginning of their relationship, Mr. West was reasonably well put together but that over time he had issues with hygiene, difficulty coming to the office and was not well put together. Towards the end, Mr. West was having mental health challenges which made it difficult for him to comply with treatment.
- [149] On assessment, he noted that Mr. West could not turn his head all the way. He saw laboured eye movements (vertical saccades). Asked about whether cannabis use could cause that

condition, he acknowledged that it could, but it would have to be in an amount that blurred a person's vision.

- [150] Mr. West told him that his car had been hit by a 30,000 pound truck and that it was like he had been hit in the back of the head with a baseball bat. He did not document a loss of consciousness.
- [151] Mr. West described impairments of the neck and jaw, cervical, thoracic and lumbar spine as well as the sacroiliac joint. Dr. Edwards testified that 80-90% of concussions resolve within weeks but Mr. West's symptoms did not. Unfortunately, he made minimal gains.
- [152] Dr. Edwards was knowledgeable, credible and reliable. He noted compliance issues and mood instability.
- [153] He saw Mr. West 65 times between 2017 and 2020 and then Mr. West's attendance dropped off. As of the date of the last visit, he continued to have concussive symptoms. His treatment focus was the impairments resulting from the mechanical injuries to the neck and back and trying to restore function and motion. A chronic strain injury like his would impact his mechanical functioning.
- [154] While the Defendants argue that his evidence cannot assist me with whether the accident caused the injuries, they concede that his evidence was credible.
- [155] Dr. Edwards could not comment on the cause of the injury because he did not see the police report. However, based on the information that he was provided, that his Jeep Cherokee was hit by a truck from behind, he stated that was a watershed moment for Mr. West.

(c) Dr. Felix Thomas (Psychologist)

- [156] Dr. Felix Thomas is a clinical psychologist who first met Mr. West on September 27, 2017 and began treatment on January 18, 2018. His opinions were based on Mr. West's self-report.
- [157] He did not know that Mr. West used cocaine or cannabis prior to the accident. He was also unaware of Dr. Randhawa's 2015 diagnosis of adjustment disorder and other previous diagnoses including the diagnosis of bipolar spectrum disorder. However, Mr. West did tell him that he had seen a psychiatrist and that there had been a diagnosis of depression and anxiety.
- [158] Dr. Thomas diagnosed Mr. West with an adjustment disorder associated with major depression and chronic pain. The motor vehicle accident was the stressor in his view. I am not concerned that there was a previous diagnosis in 2015 of an adjustment disorder because there was a different stressor in 2015.
- [159] He focussed on self-regulation meaning helping him to be mindful in his thinking and to challenge his thoughts to help him think about his emotions, engage in self-talk and question his views.

- [160] Mr. West had attendance problems. Dr. Thomas opined that he had too much on his plate. He was not able to get stabilized. He was not sleeping well. He was not able to have meals on a regular basis because of his medical conditions. Once his relationship with his girlfriend ended, he lacked social support and that contributed to a decline because there was no one able to help him consistently.
- [161] By July 2020, Dr. Thomas noted some distortions of reality. Dr. Thomas noted suicide attempts.
- [162] In his last meeting with Dr. Thomas, Mr. West was slurring his speech, he had trouble conceptualizing and was still suffering from some delusions.
- [163] The prognosis was poor. Dr. Thomas opined that there was a link between the accident and the conditions he suffered and that his prospects of returning to work were not good.
- [164] Dr. Thomas stated that Mr. West obtained greater stability once he received support from ODSP.
- [165] Notwithstanding the limits of Dr. Thomas' opinion, I value his opinions on the importance of work on Mr. West, the impact upon him of being unable to work, his condition over the time that he saw him and his impairments. Given his lack of knowledge of some of Mr. West's pre-existing challenges, his opinions on causation are of less assistance to me.
- [166] Dr. Thomas provided sessions for which he was not compensated resulting in an unpaid bill of \$5,750.

(d) Christopher Agaton (Rule 53 Expert)

- [167] Mr. Agaton is a Kinesiologist and was retained to perform a functional abilities assessment. He examined Mr. West on January 23, 2019, at the request of Plaintiff's counsel. I qualified him as an expert in kinesiology.
- [168] Mr. Agaton noted that Mr. West demonstrated a high level of effort in the assessment. He reached the following conclusions.
- [169] Mr. West has sufficient strength to perform housekeeping tasks but not necessarily home maintenance. However, certain postures and tasks would be difficult to perform.
- [170] His limitations impede his ability to perform personal care tasks without assistance. These include bathing and washing his hair without modification. He would not be able to engage in hockey or weight training because of his strength and functional limitations.
- [171] He would not be able to perform either the job of a Radiation Safety Officer or motor hand.
- [172] I found his evidence to be credible and reliable.

(e) Dr. Arnold Miners (Rule 53 Expert)

- [173] Dr. Arnold Miners is a licensed chiropractor licensed to practise in the Province of Ontario. Specialists in sports injury treatment and rehabilitation have been certified as so by the College of Chiropractic Sport Science in 2007.
- [174] He testified as a Rule 53 expert, and I qualified him as an expert in chiropractic medicine.
- [175] The Defendants argue that I should reject his opinion because he was not provided with a complete and accurate medical history. They further argue that there is a lack of credible evidence to connect Mr. West's subjective pain complaints and functional limitations to the subject accident.
- [176] Dr. Miners conducted a thorough examination using a battery of tests. He diagnosed Mr. West with the following conditions:
- a. cervicogenic headache and post-traumatic migraine;
 - b. neck pain and associated disorders Grade II (as defined by the Neck Pain Task Force and described in the scientific report of the Minor Injury Treatment Protocol project), including hypertonicity and tenderness of the semispinalis capitis muscle;
 - c. weak longus colli;
 - d. thoracic outlet syndrome (confirmed with diagnostic ultrasound) due to tight scalenes/pectorals;
 - e. lumbar sprain/strain with possible lumbar facet syndrome;
 - f. weak core stabilizers;
 - g. possible sacroiliac joint dysfunction bilaterally, the testing in this area was not conclusive;
 - h. chronic pain (myofascial pain syndrome specifically in the following muscles: left cervical paraspinals, SCM, scalene, trapezius, and levator scapulae). This is the left neck upper dorsal region. He described myofascial pain as soft tissue pain radiating myofascial tissue of the muscle, tendons and surrounding fascia;
 - i. upper cross syndrome (tight pectorals and trapezius muscles with weak scapular stabilizers and deep neck flexors); and
 - j. generalized deconditioning due to decrease in activity from the pain.
- [177] He noted some factors which would make recovering more challenging. His prognosis was guarded because of the factors which he referred to as yellow flags. These included his sleep disturbances and nervousness after the initial injury, his previous history of

psychological disturbance, his current psychosocial problems which included lack of family and social supports, his unwillingness or inability to return to active therapies and his psychological conditions.

- [178] He was challenged in cross-examination about how he could attribute these injuries to the subject accident when there were two separate subsequent incidents: the February 2017 car accident and the police altercation.
- [179] Dr. Miners acknowledged that he did not have precise information about the February 2017 car accident and the police altercation. He based his opinion on the information that he received from Mr. West that he had knee pain after the second collision and chest pain from the police altercation.
- [180] I note that he understood from Mr. West that the second accident, the February 2017 accident, impacted his knees rather than his low back.
- [181] He agreed that Mr. West would have benefited from active engagement in treatment but noted that it is common for persons with chronic pain to experience kinesiophobia and well as catastrophization of pain. Consequently, they avoid the therapy recommended but not because they are faking.
- [182] He disagreed with the proposition that Mr. West's only injuries were soft tissue injuries.
- [183] I am satisfied that Dr. Miners used appropriate testing and based on the information provided to him was able to make findings within his expertise. He properly acknowledged the limits of his expertise and the information given to him. However, due to the objective nature of his testing, I found that he was able to independently observe and make findings. It is true that the reports of pain are based on self-report.
- [184] I am confident on relying on his opinion. I believe it properly reflects an understanding that Mr. West had challenges before the accident, the nature of the accident and that the injuries correspond to it. He considered the subsequent accident and police altercation.

(f) Dr. Darryl Appleton (Rule 53 Psychiatrist)

- [185] Dr. Darryl Appleton is a psychiatrist licensed to practice in the Province of Ontario as well as the State of Florida. I found him to be a properly qualified psychiatric expert and permitted him to provide opinion evidence in the field of psychiatry. He has Board certifications in the United States for forensic psychiatry and addiction medicine. He also does work in sleep medicine after being referred by Dr. Randhawa. I found that this consult did not interfere with his ability to provide a report under Rule 53 and to give independent evidence in that capacity.
- [186] Mr. West first saw him for a sleep medicine consult on September 6, 2017. He then saw him next to prepare an opinion under Rule 53 as an expert.

- [187] Dr. Appleton diagnosed Mr. West as suffering from a Major Depressive Disorder, Severe Severity, recurrent episode. He found that Mr. West had mild difficulties with his short-term memory and this could be because of his level of depression. He opined that his depression was exacerbated greatly after his accident. He noted significant impairments to his social and interpersonal functioning and issues with his activities of daily living.
- [188] He stated that Mr. West had informed him of difficulties with activities of daily living. He also acknowledged having nightmares and flashbacks of the accident. He had difficulties with family members and others.
- [189] I accept Dr. Appleton's opinion that one should be careful about making a diagnosis of bipolar disorder where there is a history of drug abuse. Dr. Appleton noted that he did not see a documented history of mania, necessary for a diagnosis of bipolar disorder, in the self-reports. He notes that it is rare for a person to develop a bipolar disorder in their 30's. I find that there is not adequate support to find that Mr. West suffered from bipolar disorder before the subject accident.
- [190] The Defendants argue that Dr. Appleton was an advocate. I do not find that to be the case. He did defend his opinions. He was able to articulate why his opinions were unchanged by information that was not provided to him or by Mr. West's inaccurate self-reports. For example, Dr. Appleton believed that Mr. West's cocaine use was in full remission as of February, 2019 when the hospital report of March, 2019 noted that Mr. West reported that he used monthly. Dr. Appleton also believed that Mr. West was successful in school. I note that that is true. Mr. West returned to school and with some difficulty, acquired credits. That is a success for someone who had dropped out of high school. It is also true that he was not enrolled in school in the month of January, 2017. Dr. Appleton also believed that Mr. West had worked on oil rigs prior to the accident when in fact he had not worked in the oil fields for several years. However, his report clearly notes to that Mr. West came back from Alberta in 2014.
- [191] He described the impairments as severe.
- [192] Dr. Appleton's prognosis was guarded. He opined that Mr. West would have difficulty with employment.
- [193] Mr. West did not tell Dr. Appleton that he had been prescribed an antipsychotic medication prior to the accident. This also does not undermine Dr. Appleton's diagnosis.
- [194] Dr. Appleton was alive to the concerns about drug use because in his report, he noted that it would be important to ensure that random urine drug tests were conducted by his psychiatric team to ensure continued sobriety from his previous substance use. He recommended an atypical antipsychotic medication as well as other medications.
- [195] He was challenged in cross-examination for relying on Mr. West's estimation that his hypomania was limited to times when he was using cocaine. Dr. Appleton acknowledged that it was plausible that Mr. West was not being truthful with him. But I do not find that

this impacted his opinion. This was also the assessment of physicians at Lakeridge Hospital who noted that drug use could be playing a role in his psychosis.

- [196] Dr. Appleton stated that he was asked to assess Mr. West's current condition and not do a full psychiatric "work-up". He also talked about the importance of testing and statistical information grounding his opinion.
- [197] Asked about the use of cannabis, Dr. Appleton said he does not see a use for it for depression, but it can assist with insomnia and pain. He agreed that some people seek out their own treatment, but he would not prescribe cannabis.
- [198] I found Dr. Appleton's assessment to be of assistance. His evidence withstood cross-examination despite the limits it exposed on the state of his knowledge about his pre-accident history. His evidence aligns with what was described by the lay witnesses as well as the evidence of Dr. Thomas, Dr. Ko and Dr. Randhawa.

(g) Dr. Gordon Ko (Rule 53 Expert Psychiatry)

- [199] Dr. Gordon Ko is a psychiatrist, and I qualified him as an expert in that area and permitted him to provide opinion evidence in accordance with Rule 53. He was also involved with Mr. West in providing care shortly after his accident. His limited role in that latter capacity did not concern me about his ability to fulfil his duty to the Court to provide an independent and impartial report.
- [200] His assessment relied in part on the work of Dr. Miners, and he agreed with Dr. Miners' diagnosis. He diagnosed Mr. West, among other things, with cervicogenic headaches, post-traumatic migraine, possible post-concussional disorder, chronic pain syndrome and vasculogenic thoracic outlet syndrome. He found these to be serious and permanent impairments in the cervical and lumbar spine and upper left extremity. He found that chronic pain is difficult to treat with poor prognosis and limited improvement. He will continue to suffer pain and limitations in his function.
- [201] His therapeutic recommendations included education, interventional therapy, botox injections, a strengthening program, a stretching and strengthening program to address the functional disorders, a neuropsychological assessment as he suspected a mild traumatic brain injury, vocational assessment and rehabilitation, medication (Tizanidine), topical lidocaine and potential supplementation with testosterone recommended, among other things.
- [202] He found that Mr. West scored high on the Tampa Scale for Kinesiophobia which examines fear avoidance, fear of work-related activities and fear of movement-injury. Based on this score, Dr. Ko recommended further investigation and psychological support.
- [203] Dr. Ko was challenged in cross-examination about the information that he did not receive from Mr. West about his pre-accident life. He acknowledged some deficits but I find that they did not affect his opinion.

- [204] Dr. Ko described that the gold standard test is doing the Facet Joint Block testing. This involves injecting medication into the spine. Mr. West would not consent to this testing.
- [205] Questioned in cross-examination about his pre-accident condition, and his post-accident condition, Dr. Ko said that Mr. West was like a fragile egg and that the car accident was a tipping point. He said that that type of impact caused by being rear ended was a major player. He had mental health conditions but he was physically not too bad, referring to his ability to work in the oil fields and play sports. The Defendants' counsel challenged him on that assertion suggesting that he may have only worked a total of eight months in the oil fields.
- [206] Dr. Ko was prepared to rely on his sports activity as support for Mr. West being physically unwell. He was told by Defendants' counsel that that history is unclear. I agree that it is. However, I pause to note that the Court heard clear evidence that Mr. West was able work out at the gym and swim and do indoor and outdoor home maintenance prior to the accident.
- [207] Dr. Ko remained of the view that the January 16, 2017 accident was the major player and the tipping point probably leading to his injuries.
- [208] I accepted his evidence. I found that he properly acknowledged the limits of his opinion.
- (h) Marla Tennen (Rule 53 Expert)
- [209] Marla Tennen is a registered nurse. I permitted her to give opinion evidence on the cost of future care. I will discuss her evidence below when I examine that issue.
- (i) Saqib Durrani (Rule 53 Expert)
- [210] Saqib Durrani is an economic consultant with a M.Ag. Econ. and an M.B.A. I permitted him to give opinion evidence on the issue of past and future income loss. I will consider his evidence when I address that issue.
- (j) Dr. Michael Boucher (Rule 53 Pain Specialist)
- [211] Dr. Michael Boucher is a physician licensed to practice in the Province of Ontario and other places. He has a specialty in family medicine after which he completed two years of a residency in Orthopaedic Surgery. He has had additional formal training in chronic pain and states that the College of Physicians and Surgeons of Ontario have granted him the authority to focus his practice on chronic pain management. He has been connected to teaching hospitals and is presently the Medical Director of Burlington Pain Care.
- [212] I found him to be an expert in chronic pain and gave permission for him to give evidence in this area.
- [213] He completed a detailed and thorough review of Mr. West's past medical, psychological, medication, social and vocational and educational history.

- [214] He completed standardized testing recommended by the American Medical Association Guide to the Evaluation of Permanent Impairment. The tests – the Multi-Dimensional Assessment Tools - completed by Mr. West revealed adverse impact by pain symptoms, complete disability on the Neck Disability Index, severe disability on the Oswestry Low Back Pain Disability Questionnaire, complete disability on the Headache Disability Index, as well as other testing revealing disability, including one area where it would not be expected (the lower limbs).
- [215] On the pain catastrophizing scale, Dr. Boucher noted that:
- Mr. West's total score (100th percentile) indicated clinically significant levels of catastrophizing. His scores on the Rumination (100th percentile), Magnification (100th percentile), and Helplessness (100th percentile) subscales were all significantly elevated in comparison with other individuals who experience chronic pain. These results suggest that Mr. West has an exaggerated negative orientation towards his experience of pain, in that he is prone to ruminate about and amplify his pain and respond to it with a sense of helplessness because of his thinking patterns. Mr. West's responses on the PCS yielded an elevated aggregate score, indicating the presence of dysfunctional and catastrophic thoughts in response to his pain that place him at increased risk to develop a somatic symptom disorder.
- [216] Dr. Boucher examined Mr. West on February 2, 2023. On examination, Mr. West complained of left-sided neck pain, upper back pain, left trapezius pain, headache, depressed mood, anxiety and difficulty with sleep.
- [217] On assessment, he found no objective evidence of physical impairment. He found that Mr. West demonstrated functional range of motion of his spine and full range of motion in the extremities. He suspected that the perceived levels of pain may be related to his psychological impairment and deferred further comment to a psychiatrist.
- [218] Dr. Boucher found no objective physical impairments. He attributed symptoms largely to pre-existing mental health and substance use issues. He did not set out how the standardized testing integrated with his observations.
- [219] He concluded that Mr. West sustained primarily soft tissue injuries and that he does not demonstrate any significant objective evidence of ongoing impairment. He had functional range of motion of his spine. There was no soft tissue tone in any area of chief complaint. Dr. Boucher concluded that the perceived level of physical pain are related to a psychological impairment.
- [220] I had some difficulty with Dr. Boucher's evidence for three primary reasons.

- [221] The first relates to his physical observations and range of motion testing. He has a degree in kinesiology but I did not qualify him as an expert in such. He expressed opinions about orthopaedic and neurological functions. I am cautious about his evidence in those areas.
- [222] The second relates to his reasoning about chronic pain. He did not address the diagnostic criteria for chronic pain in his report. In cross-examination, he was asked to set out the criteria for chronic pain or evidence in chief. He relied on the 6th Edition of the AMA Guidelines which he states are generally accepted. Under those guidelines, three or more of six criteria are required. The criteria are: use of prescription drugs beyond what is recommended, excessive dependences on health care providers, physical deconditioning (as some persons will no longer be active because they are afraid, withdrawing from the social milieu), failure to restore pre-injury function and the development of social sequelae (i.e. Anxious, becoming depressed). He said that Mr. West's conditions do not support a diagnosis because they were present before the accident.
- [223] He said that there is a history of prescription drug use prior to the accident. He said that he was seeking Percocet from clinicians before the accident, he did not see evidence of deconditioning. He did not seem aware of Mr. West's physical strength pre-accident and believed that he was withdrawn before the accident.
- [224] I have had the benefit of hearing from Mr. West, his mother, Ms. Robinson and others who support the changes connected to the criteria. Dr. Boucher did not have that benefit. He overemphasized aspects of the health history. I am not confident in his conclusions. I also would have liked him to directly address the diagnostic criteria in his report.
- [225] The other challenge I had with Dr. Boucher's evidence is that he exaggerated in a significant way in describing the second accident. He stated that Mr. West "wrapped his car around a pole" when there is no such evidence. Moreover, it is colourful language and unmeasured language ill-suited for an independent expert with a duty to the Court.
- [226] In the end, I am not confident in relying on his opinion.
- (k) Dr. Andrew Holland (Rule 53 Life Care Planner)
- [227] Dr. Andrew Holland testified as a Rule 53 expert. He is a life care planner. I will address his evidence under the cost of future care.
- (l) Dr. Allan Peterkin (Rule 53 Psychiatrist)
- [228] Dr. Allan Peterkin was qualified as an expert in psychiatry.
- [229] Dr. Peterkin did a thorough review of Mr. West's health history.
- [230] He said that Mr. West acknowledged behavioural problems as a child and that he was truant. He was placed in group homes. He acknowledged that he was angry and that he would get into fights and that he vandalized. None of this was put to Mr. West or his mother when they testified.

- [231] He told Dr. Peterkin that after the accident he became addicted to pain killers and had made multiple suicide attempts.
- [232] Mr. West acknowledged a long-term problem with anger.
- [233] Dr. Peterkin believes that the cannabis use of upwards of 25 joints per day was not therapeutic and might make his condition worse including increasing his anxiety and contributing to a psychosis.
- [234] He saw Mr. West as a person with considerable vulnerabilities pre-accident.
- [235] Dr. Peterkin stated that Mr. West was a challenging person to interview. He was defensive, sarcastic and irritable. This aligns with my observations. His speech was slurred, and he acknowledged that he had smoked marijuana before the assessment. He appeared intoxicated. His thought process was disorganized and vague in his answers, sometimes giving contradictory answers.
- [236] Dr. Peterkin felt that he had withheld details and considered that he might exaggerate symptoms.
- [237] At the time of the assessment, Mr. West was not taking any prescription medications including psychotropic medications.
- [238] Dr. Peterkin noted that there had been many assessments and that there was not a lot of concurrence of opinions which spoke to the high complexity of the case.
- [239] He stated that Mr. West used and misused cannabis. He described his problem with cannabis as chronic. He stated that Mr. West told him that he was smoking 25 joints per day or more. Dr. Peterkin stated that this was misuse.
- [240] Dr. Peterkin diagnosed Mr. West with Somatic Symptom disorder with Predominant Pain, marijuana use disorder with a history of polysubstance use disorder, Adjustment disorder with depressed mood and anxiety, chronic Mood Disorder Not Otherwise Specified (with multi-factorial causes including substance use) and Obstructive Sleep Apnea (untreated).
- [241] He agreed that a bipolar disorder was difficult to make out if the person was using substances.
- [242] Dr. Peterkin also found that Major Depressive Disorder should be ruled out, as well as ADHD and Bipolar Disorder and well as personality disorders.
- [243] Dr. Peterkin found that he does suffer from a psychiatric impairment: chronic pain, marijuana use disorder, a chronic personality disorder and a chronic (pre-accident) mood disorder. However, he noted that his impairment was only a minor causative piece. He is significantly impaired but not solely or significantly because of the accident. He states that his chronic pain is strongly linked to his pre-existing psychiatric problems. He believed that Mr. West would be able to work in some capacity if his marijuana use disorder was

fully addressed along with management of his chronic mood and pain disorders. He said that the odds are against him.

- [244] His prognosis was guarded, not because of the accident but because of the poor prognosis indicators, including what he described as a chaotic childhood, rage problems, substance problems, all of which were pre-accident. His present situation is complicated in his view by his refusal to take treatment. He states that there are many other causes more significant of his present situation than the accident. Mr. West had a “chronically stressed” existence.
- [245] Dr. Peterkin opined that he may need lifelong care, but not because of this accident. He thought Mr. West would benefit from a shared care model with a psychiatrist and family doctor working together and abstinence from substances.
- [246] In cross-examination, the suggestion that Mr. West was a “fragile egg” and the accident a tipping point. Dr. Peterkin said that there were multiple complications engendered by his decisions that were more significant than the subject accident. In his view, his refusal of care was more significant than the subject accident.
- [247] I found Dr. Peterkin’s analysis helpful in terms of his present psychiatric issues. I have some hesitation about accepting it fully. He does not have the benefit of the collateral information from the evidence that the Court heard. Neither Ms. Robinson nor Ms. West described Mr. West as an angry and aggressive person before the accident. Mr. West did not have the chance to respond to some of the early childhood issues that Dr. Peterkin relied upon. That is not the fault of Dr. Peterkin.
- [248] I also cannot accept his conclusion that Mr. West refused care. It is true that he has not taken antipsychotic medication and that he refused nerve blocks by injection. However, in many cases, he was refused care by clinicians because of the anger and personality changes that emerged post-accident. So, while I can find that his diagnostic opinions on Mr. West’s present impairments are sound, I find his opinions on causation less convincing, because of his overly negative view of Mr. West’s pre-accident life with which I disagree. Not only do I have the benefit of hearing from Mr. West’s mother and former girlfriend, I also have the benefit of Dr. Randhawa who witnessed not only the health problems, but a personality change. Dr. Edwards also detailed a decline as Mr. West struggled with post-accident pain.
- [249] In short, I accept his opinions on diagnosis and the existence of impairment.

(m) Dr. Freedman (Neuropsychologist)

- [250] Dr. Freedman was called by the Defendants as a Rule 53 expert to provide evidence in neuropsychology, and I qualified him to give opinion evidence in that field.
- [251] Dr. Freedman assessed Mr. West on July 11, 2024. He retired from practice in 2024. Dr. Freedman last worked in a hospital setting treating patients in 2006. Since that time, his work has been assessment work doing private assessments. In that work he also diagnoses the person being assessed and sometimes makes recommendations. In his estimation, 95% of the assessments have been at the request of defence, and 5% plaintiff side work.

- [252] Dr. Freedman asked Mr. West about his memory prior to the accident, during the accident and following the accident. Mr. West recalled leaving school and the car he was driving. He said following the accident he thought he was dying three times. His memory of the accident is that the rear windshield broke and the airbags deployed. He also told Dr. Freedman that he did not recall the collision and that there was a loss of consciousness. He recalled that an ambulance came. He recalled calling his girlfriend who came and took him to the hospital.
- [253] Dr. Freedman asked those questions because the way to assess a traumatic brain injury is to examine the acute clinical injury at the time of the trauma. One of the criteria is whether the person experiences a loss of consciousness and whether they were confused. Confusion comes from intercranial trauma. That confusion is post-traumatic amnesia. A person with post-traumatic amnesia would not recall events immediately post accident. There is also retrograde amnesia. They would have no memory of the accident and the events just prior to it.
- [254] He said he would normally place great emphasis on the ambulance call report. None was available. He said if Mr. West was having symptoms of a traumatic brain injury, he would have at least been confused. He noted that the emergency room doctor did not find deficits. His pupils reacted to light appropriately and that there was no slurred speech, for example.
- [255] While Mr. West told him there was a loss of consciousness, I note that he has told others that he did not lose consciousness.
- [256] Dr. Freedman stated that he would not diagnose a traumatic brain injury based on self-report made after some time after the accident. He would need to have corroboration from the medical record. He did not have that corroboration. He concluded that there was no evidence of traumatic brain injury based on testing and medical records. He was critical of Dr. Randhawa for diagnosing a mild TBI (i.e. concussion) without reference to criteria. The only evidence are cervical and thoracic sprains.
- [257] Dr. Freedman noted impairments in processing speed and memory impairments. However, he did not find evidence of a brain injury.
- [258] In short, he found no neurocognitive deficits because there was no neurotraumatic injury to the brain. He found cognitive abnormalities but, in his opinion, these cannot be referred to as a traumatic brain injury.
- [259] Dr. Freedman's opinion made sense but I have some reluctance in relying on it because he has not treated patients with brain injury since 2006 and only assessed, diagnosed and made recommendations for persons alleged to be suffering from brain injury after the fact with 95% of that work being on the defence side.

IV. Legal Analysis

- [260] Having reviewed the bulk of the evidence relating Mr. West's pre-accident life, the accident, and his post-accident life, I now turn to the legal principles relating to causation

and then address my findings on the evidence. I will take the same approach for damages and then the threshold question.

A. Causation

- [261] The test for causation was set out by Chief Justice McLachlin in *Clements v. Clements*, 2012 SCC 32, [2012] 2 S.C.R. 181, at para 8:

The test for showing causation is the “but for” test. The plaintiff must show on a balance of probabilities that “but for” the defendant’s negligent act, the injury would not have occurred. Inherent in the phrase “but for” is the requirement that the defendant’s negligence was necessary to bring about the injury -- in other words that the injury would not have occurred without the defendant’s negligence. This is a factual inquiry. If the plaintiff does not establish this on a balance of probabilities, having regard to all the evidence, her action against the defendant fails.

- [262] In *Sacks v. Ross*, 2017 ONCA 773, at paras. 47 – 48, Justice Lauwers describes a three-step reasoning process to assess causation. First, decide what likely happened. Second, decide what likely would have happened if the defendant had not breached the standard of care. Justice Lauwers described this as an “imaginative reconstruction of reality”. There is a third step to allocate fault among negligent defendants.
- [263] On the second step, if I infer from the evidence that Mr. West would have likely been injured in any event, regardless of what these Defendants did or failed to do in breach of the standard of care, then these Defendants did not cause the injury. However, if I infer from the evidence that Mr. West would not likely have been injured had this accident not occurred, then the “but for” test is satisfied.
- [264] This means I must first decide what happened. I must then determine what his life would have been like without the accident happening. This necessarily involves examining Mr. West’s life before the accident and the other harm that happened to Mr. West after January 16, 2017, relating to events that these Defendants were not parties to.
- [265] If the Plaintiff proves on a balance of probabilities that the January 16, 2017 accident caused harm, either on its own or in combination with the other bad things that happened after it, then causation will be proven. In short, I must examine whether the Plaintiff has proven on a balance of probabilities that the Defendants’ breach of the standard of care, alone or in combination with other factors, was a necessary factor in bringing about the harm to Mr. West.
- [266] It is well understood that the Defendants must take Mr. West as they find him including any pre-existing vulnerability. This is the rule referred to as the thin skulled plaintiff. A defendant is liable for a plaintiff’s injuries even if the injuries are unexpectedly severe due to a pre-existing physical or mental health condition. This is commonly known as the “thin skull” doctrine. *Janiak v. Ippolito*, [1985] 1 SCR 146.

[267] However, where a plaintiff is in a state of deterioration before the accident and would have continued to deteriorate without the accident, another doctrine applies. This is commonly and awkwardly known as the “crumbling skull” doctrine. In such a case, the defendant is not required to put the plaintiff in a better position than he would have been, had the accident not occurred. As noted in *M.B. v. 2014052 Ontario Ltd. (Deluxe Windows of Canada)*, 2012 ONCA 135, at para. 36:

If there is a “measurable risk” that the condition suffered by the plaintiff would have affected the plaintiff in the future [page361] regardless of the defendant’s tortious act, the defendant will be liable for the effect of his act on the degenerative process. However, the defendant need not compensate the plaintiff for any debilitating effects of the pre-existing condition that would have occurred in any event: see *Athey*, at para. 35.

[268] In short, if the plaintiff’s pre-existing condition would have caused harm and/or manifested itself in any event, regardless of the defendant’s actions, the defendant need not compensate the plaintiff for any debilitating effects of the pre-existing condition which the plaintiff would have experienced anyway.

[269] These two concepts distinguish a plaintiff with health vulnerabilities from a plaintiff whose decline or deterioration in health is inevitable.

[270] If Mr. West proves causation on the “but for” test, that is, the accident was a necessary cause of the damage, the Defendants are responsible for injuries that are unexpectedly severe given the pre-existing vulnerability. In short, even if the average person might not have suffered the type of injuries suffered by Mr. West, the Defendants will be liable for his losses.

[271] After determining factual causation, I then examine the question of legal causation.

B. Findings

(a) Pre-Accident Health

[272] I find that the accident was a watershed moment. Before the accident, Mr. West was getting by. He had a purpose. He had mental health challenges, but they did not substantially interfere with his personal relationships. These were anxiety and depression and some drug use, marijuana daily and the occasional use of cocaine. The medical records reflect a concern both that Mr. West was dependent on them and that he would develop an addiction. There were some crisis hospitalizations. There is one reference to past substance abuse. However, he was able to obtain some credits in 2016 when he returned to school.

[273] Mr. West did not have stable mental health. He struggled when he was working in Alberta and British Columbia, and when he returned when family members were ill. The death of relatives affected him greatly.

- [274] There is ample evidence that Mr. West used drugs. Mr. West's pre-accident substance use did not impair his ability to have relationships and did not appear to interfere with his ability to return to school. I reach this conclusion through Ms. Robinson's evidence and Ms. West's evidence. Ms. Robinson was aware of his daily use of marijuana.
- [275] There is some evidence of an addiction at points predating the accident. I draw a distinction between substance use and a diagnosed condition and/or dependency.
- [276] In her August 21, 2014 note, Dr. Racicovschi wrote that in the short admission Mr. West improved quite fast "which might give us an idea that the cause of his psychosis is marijuana abuse or other drugs or the patient was in withdrawal". This is not conclusive. Her diagnosis was Psychosis NOS which I understand to be Psychosis Not Otherwise Specified.
- [277] However, in 2016, there was a deterioration. Dr. Racicovschi wrote in her October 7, 2016 letter to Dr. Randhawa:

He has normal energy, no problems with anger, no suicidal ideation, no homicidal ideation and no agoraphobia. For his anxiety he was taking up to 2.5 mg of Clonazepam each day. He also resorts to using cannabis, smoking up to 1 gram per day.

In addition to cannabis, he also has a history of cocaine usage, although he has not used any in the past 8 months. He also has a history of alcohol abuse, although he states that his last drink was over 3 months ago.

...

In summary, Douglas West is a 31-year-old man with bipolar spectrum disorder, generalized anxiety disorder and is currently in a depressive phase. He also has a history of substance abuse. We also need to rule out attention deficit hyperactivity disorder. We have thus give him a test for ADHD to complete and return back in follow-up. Douglas has also been educated about cannabis rebound anxiety and depression and we have advised him to make a serious attempt at stopping cannabis usage. We also warned him of the dangers of benzodiazepine addiction. To treat his mood disorder, we have prescribed him Olanzapine 2.5 mg p.o. q.h.s. as well as for his anxiety, zopiclone 7.5 mg p.o. q.h. s. p.r.n. which should help him to sleep. We have also prescribed him Clonazepam 2 mg p.o. daily p.r.n.

- [278] The letter reflects that at that time, he was reporting that he had friends that he hung out with.

- [279] From that note it appears that there is a concern about alcohol abuse in the past and a concern about cocaine use. Dr. Racicovschi did not make a diagnosis of substance abuse disorder or addiction. Mr. West told her that he was having difficulty focussing on school.
- [280] I note that Mr. West's statement that he had used cocaine in the last eight months is undermined by the June 29, 2016 note where he reported to Lakeridge Health with a rash after using cocaine, which was just over three months before Dr. Racicovschi's October 7, 2016 notes.
- [281] I can conclude that pre-accident, Mr. West was experiencing anxiety and depression as well as insomnia. He also used drugs and there has been concerns about dependency on them. The mental health admissions to hospital were episodic and time limited. There were no lengthy admissions on the evidence before me.
- [282] Dr. Racicovschi diagnosed Mr. West with a bipolar disorder in 2016. However, I accept Dr. Appleton and Dr. Peterkin's opinion that Mr. West's drug use makes the diagnosis difficult to make.
- [283] In short, I find that Mr. West had challenges which made him vulnerable, but they were not disabling. Rather than deteriorating at the time of the accident, he had a relationship. He was able to engage with Ms. Robinson's child. He did home maintenance and chores and went to the gym. It also appears that he had friends. He had successfully completed some schooling in 2016.
- [284] I also rely on the evidence of Dr. Randhawa, Dr. Ko and Dr. Appleton and accept their analysis on the state of Mr. West's pre-accident health.
- [285] The Defendants argue that many of the Plaintiff's experts were not provided with credible information upon which to make their diagnoses and that this undermines their opinion. I agree that Mr. West did not reveal the scope of the challenges that he faced pre-accident. He painted a much rosier picture of the time immediately before the accident and I find that he believes life was better before the accident. Records undermine the reliability of his depiction of his life pre-accident. He was struggling with mood (either a bipolar disorder or depression) and anxiety. He was also using drugs. That did not meaningfully impair his social relationships and his ability to continue in school. I find this because he did return to school and persevered.
- [286] However, I find that the rosier picture painted by Mr. West does not undermine the expert's opinions. They were able to distinguish the challenges he faced then with his challenges now, Dr. Ko using the example of him being a fragile egg.

(b) Accident

- [287] I find that Mr. West was driving a Jeep Cherokee that was hit from behind with such force that the rear windshield shattered, the bumper was dented in, the headrest airbag deployed, and Mr. West hit the car in front of him.

(c) Injuries Sustained

- [288] Mr. West gave a vivid description of how he felt. Ms. Robinson described him as being confused. He was assessed at Lakeridge Health and no confusion was noted. However, four days later, on January 20, 2017, he saw Dr. Lee who worked with Dr. Randhawa reporting pain, fatigue and dizziness and Dr. Lee prescribed Gabapentin for the pain.
- [289] On February 1, 2017, Dr. Randhawa assessed a whiplash injury and noted mild concussion. Mr. West complained of neck and shoulder pain. Dr. Randhawa observed decreased flexion neck, trapezius tenderness, and cervical tenderness.
- [290] The Defendants argue that the injuries suffered by Mr. West were low level soft tissue only and that he had significant pre-existing problems. They assert that he was not working before the accident and that he was likely to be on social assistance for the rest of his life. They assert that he failed to adhere to treatment recommendations and failed to mitigate his damages. They seek to reduce any award of damages. I begin by examining the law regarding causation and threshold because it helps focus the analysis of the evidence.
- [291] However, the references to addiction and his treatment for it at Pinewood are post-accident. For example, his April 2019 admission to Lakeridge Hospital referenced issues regarding addiction and substance abuse.
- [292] However, the pre-accident and post-accident Mr. West, on the evidence before me, are starkly different. This comes from the totality of the lay evidence and from Dr. Randhawa. Those who knew Mr. West before and after the accident spoke clearly about the difference.
- [293] Dr. Randhawa noted post-concussive symptoms. Dr. Freedman described a concussion as a mild traumatic brain injury but said one could not be diagnosed on the available evidence because they could not be diagnosed on self-report.
- [294] The January 2017 accident was the significant event. All the evidence that I have is that the second accident and the altercation with the police did not cause his current presentation.
- [295] I find that he did suffer whiplash, neck and back pain, that he became unable to work or go to school and that he had difficulty with even the most basic things. He exhibited symptoms of a concussion on the evidence of Dr. Randhawa.
- [296] I find, based on Dr. Appleton's, Dr. Thomas' and Dr. Randhawa's evidence and Dr. Racicovschi's March 7, 2018 note, that the accident exacerbated Mr. West's depression. I also find that the accident was a necessary factor for this worsening and that it would not have worsened had the accident not occurred.
- [297] I appreciate that many of the assessments at trial are dated. Dr. Appleton was guarded in his prognosis. Many of the symptoms he complained of to his clinicians were issues that he complained of in Court.

- [298] I also find based on the evidence of Mr. Agaton and Dr. Ko that because of the accident Mr. West suffers from diffuse chronic pain syndrome, Myofascial Pain Syndrome specifically in the left cervical paraspinals, SCM, scalene, trapezius, and levator scapula muscles. Dr. Thomas opined that Mr. West suffers from chronic pain, as did Dr. Racicovschi in her March 8, 2018 note. I find that these conditions have limited his ability to perform basic household activities and his activities of daily living.
- [299] Dr. Ko also diagnosed that the following conditions were the result of the accident:
- a. Possible Post-concussional disorder as per Ontario Neurotrauma Foundation Guideline on MTBI and Persistent Symptoms. (requires psychiatric or neurological confirmation)
 - b. Neck
 - i. Neck pain and associated disorders Grade II (as defined by the Neck Pain Task Force and described in the scientific report of the Minor Injury Treatment Protocol project), including hypertonicity and tenderness of the semispinalis capitis muscle.
 - ii. Possible cervical facet syndrome and neuropathic pain syndrome.
 - iii. Weak longus colli.
 - c. Thoracic
 - i. Vasculogenic thoracic outlet syndrome (confirmed with diagnostic ultrasound) due to tight scalenes/pectorals.
 - d. Lumbar
 - i. Lumbar sprain/strain with possible lumbar facet syndrome.
 - ii. Weak core stabilizers.
 - e. Sacroiliac
 - i. Possible sacroiliac joint dysfunction bilaterally.
 - f. Chronic Pain

C. Did the Accident Cause the Injuries?

- [300] Mr. West had no complaints of any physical injuries (i.e. neck, shoulder and back pain) prior to this accident.

- [301] The accident caused a personality change noted by those around him and his clinicians, including Dr. Racicovschi and Dr. Randhawa. He lost his ability to cope with people. He lost his ability to look after himself.
- [302] These injuries to his mental and physical health as well as his personality were not the inevitable result of his pre-existing health problems. I find on a balance of probabilities that the accident was the cause of his physical injuries, his personality change, and the exacerbation of his mental health problems. I also find that these injuries were reasonably foreseeable.
- [303] I find that before the accident, Mr. West was working towards obtaining his high school equivalency. However, he was not actually in school on the day of the accident. He had been enrolled in adult education for much of 2016. This was derailed in part in August, 2016 when he had cysts and again in September 2016 when he was hospitalized for mental health reasons. He resumed classes, albeit in a hybrid fashion, in March, 2017 after the accident and managed to complete another course.
- [304] The Defendants argue that because he returned to school in March 2017 and obtained a credit that I can infer that it was subsequent events that derailed his life. I do not accept this. Rather, the evidence both of Ms. Robinson and Mr. West, together with contemporaneous medical records demonstrate that the January 16, 2017 accident was the significant event.
- [305] Is it fair for Mr. West to say that he was “in school” at the time of the accident even though he was not enrolled in a course at that moment? I think it is. He was motivated to move forward with his career. He attempted to enrol in a hybrid model. Many people consider themselves to be in school even when they have a lengthy break between terms. I find it is fair for him to say that he was in school.
- [306] In my view, the drastic change in his social functioning is more likely than not the direct cause of the accident and that would not have occurred but for the accident.

(d) Mitigation

- [307] The Plaintiff is expected to mitigate his losses. The Defendants argue that Mr. West’s present challenges arise from a refusal or failure to take all reasonable and/or proper steps to mitigate his accident-related injuries and impairments and the associated damages. The onus is on the Defendants to prove that on a balance of probabilities.
- [308] To prove a failure to mitigate, the Defendants must establish on a balance of probabilities that Mr. West was unreasonable in refusing to undertake the recommended treatment. This is determined objectively by determining what a reasonable person would do in the circumstances faced by the Plaintiff. See: *Janiak v. Ippolito*, *supra*, at para. 34 and *Jespersion v. Karas*, 2019 ONSC 5841, at paras. 267 – 268.
- [309] If failure to mitigate is proven, the damages are reduced accordingly.

- [310] The Defendants argue, relying on Dr. Peterkin's evidence, that Mr. West unreasonably refused every treatment modality and refused all treatment except Percocet which was contradicted.
- [311] The facts do not support this. Mr. West refused an injection. Reasonable persons refuse needles. However, he sought both chiropractic services and psychological services. He ran out of money. He took Gabapentin for pain.
- [312] I find that the Defendants have not proven that Mr. West failed to mitigate because they have not proven on the first prong, that Mr. West unreasonably refused treatment.

D. Quantifying Damages

- [313] In assessing damages, the Plaintiff is entitled to a monetary award sufficient to return them to the position that they would have been in had there been no accident. The focus is on actual losses and no more. This compensation occurs regardless of the measure of the Plaintiff's personal vulnerability: *Cunningham v. Wheeler*, 1004 CanLII 120, at para. 5.

(a) General Damages

- [314] The Plaintiff seeks damages for non-pecuniary losses which are awarded for pain, suffering, disability, inconvenience, and loss of enjoyment of life. I must fix a sum that is fair and reasonable. I must look at what he has suffered already and what he is expected to experience in the future.

(i) Positions of the Parties

- [315] Mr. West seeks damages for pain and suffering in the range of \$200,000 to \$225,000 considering the nature of the injuries, the impact and permanence of them, his psychological and emotional distress and his age.
- [316] The Defendants argue that damages should be awarded for soft tissue injuries only. As I have rejected that Mr. West's injuries were limited to soft tissue injuries, I find the cases put in support of their position to be of little use to me.

(ii) Analysis

- [317] I agree that there are cases that award damages in the range proposed by the Plaintiff. These cases have similar injuries and post-accident problems. For example, *Taylor v. Zents*, 2024 ONSC 166, at para. 409, where \$250,000 was the award with similar injuries, *Graul v. Kansal*, 2022 ONSC 1958, at para. 562 (\$225,000) among others. The low-end of the range of cases, with inflation, is \$167,000 to \$250,000.
- [318] In my view, this case is closer factually to *Rolley v. MacDonell*, 2018 ONSC 6517, at para. 418. In that case, the plaintiff was 54 years old at the time of the accident and 60 years old at the time of the trial with several pre-existing conditions including chronic pain. However, he managed with support. Among other things, the plaintiff developed somatic

symptom disorder as well as a recurrence of major depression. The Court awarded \$142,500 after deducting 25% which the plaintiff submits is \$167,183.41 in 2024 dollars.

- [319] Mr. West was younger at the time of the accident but his challenges both before and after the accident were similar. Mr. West's losses weighed on him – "I died in that accident".

(iii) Conclusion on Non-Pecuniary Losses

- [320] I assess Mr. West's general damages for pain, suffering and loss of enjoyment of life at \$200,000.

(b) Past Economic Loss

- [321] Past economic loss is established on a balance of probabilities. However, future economic loss involves looking at how Mr. West's life will unfold from this point forward. It is determined by whether there is a real and substantial likelihood of the event: *Beldycki Estate v. Jaipargas*, 2012 ONCA 537, paras. 73 – 75.

(i) Positions of the Parties

- [322] Mr. West seeks damages for past income loss in the range of \$222,768 to \$354,778 based on different scenarios.
- [323] The Defendants argue that there is no credible evidence that Mr. West suffered any income loss. They further argue that if I find income loss, I should rely only on Ms. McCannell's evidence over Mr. Durrani's evidence which they assert is not legally supportable.
- [324] This is based on Mr. Durrani's evidence that average his historical earnings from 2008 to 2014 which is \$35,933 in 2025 dollars.

(ii) Evidence

- [325] Mr. West could not provide reliable evidence on the timing of what he did and the hours that he worked. His evidence does establish on a balance of probabilities that pre-accident he was able to work on and off in general labour and kitchen jobs.
- [326] Mr. West's last job was at Trinidad Drilling which he held from September 5, 2014 to December 1, 2014. During that period, he worked 399 regular hours and 390 overtime hours.
- [327] I have noted that Mr. West did not work in 2015. In his Ontario Works file, there is a November 16, 2015 note that he had started the pardon process and Mr. West testified that he intended to secure a pardon.
- [328] His income tax returns establish his net income. Table 1 of Mr. Durrani's report sets out the income earned in those years, and I reproduce it below. It is accurate based on a review of the source income tax returns that are in evidence.

Mr. West's Income History

	Line	2008	2009	2010	2011	2012	2013
		(\$)	(\$)	(\$)	(\$)	(\$)	(\$)
Employment income	101	28,449	30,196	12,801	9,558	6,665	25,769
EI benefits	119						10,521
Social assistance payments	145		610	1,255	2,944	7,528	2,179
Total	150	<u>28,449</u>	<u>30,806</u>	<u>14,056</u>	<u>12,502</u>	<u>14,193</u>	<u>38,469</u>

	Line	2014	2015	2016
		(\$)	(\$)	(\$)
Employment income	101	45,494		
Social assistance payments	145	8,175	10,550	9,517
Total	150	<u>53,669</u>	<u>10,550</u>	<u>9,517</u>

(1) As per Canada Revenue Agency documentation for the years 2008 to 2016.

[329] Mr. Zervas prepared a table comparing Mr. West's income to that of Statistics Canada "Employee Wage by Industry" which provides the median hourly rate for males (15 years and over) working on a full-time basis in Alberta on the oil rigs. The comparison is instructive:

Year	Median Hourly Rate	Estimated Annual Earnings (based on 40-hour work week)	Mr. West's Reported Employment Earnings:
2008	\$32.70	\$68,016.00	\$28,449.00
2009	\$35.58	\$74,006.40	\$30,196.00
2010	\$37.08	\$77,126.40	\$12,801.00
2011	\$37.00	\$76,960.00	\$9,558.00
2012	\$40.68	\$84,614.40	\$6,665.00
2013	\$40.87	\$85,009.60	\$25,769.00
2014	\$41.00	\$85,280.00	\$45,494.00

[330] I accept, based on Diamantis Zervas' testimony, that we can discern from these numbers that Mr. West was not working full-time in the oil field during these years. It is also a bit of a mystery why Mr. West collected Employment Insurance in only one year.

[331] The income tax returns establish that Mr. West's pre-accident earnings were inconsistent. He was at times on social assistance. He had no reportable income in the two years prior to the accident.

[332] At the time of the accident, however, he was attending school, and he was achieving, albeit with difficulty.

- [333] I find he would have completed his high school credits in 2017 and gained entry to the Radiation Safety Program as he hoped in the fall of 2017. It was a two-year program. I find that Mr. West was not capable of sustaining an accelerated program while being away from his social supports in 2017 before the accident occurred.
- [334] Is there a real and substantial likelihood that he would have succeeded in the program? I am not convinced, despite the real work Mr. West put into the attempt. I find that he would have faced barriers. He would need to secure a pardon. His drug use would present a problem. While there was a real possibility that he could have been a Radiation Safety Officer, I find that because of these challenges, it was not a substantial one.
- [335] However, I find that there is a real and substantial likelihood that he would have returned to work but for the accident. He overcame some losses (the death of his grandfather, his uncle and his aunt and his mother's accident) to return to school. I find that he would have worked but with some instability. He had worked from a young age. This was off and on in manual jobs: in kitchens, in the oil fields, and for a moving company. I find that he would have worked again. I also find that based on his inconsistent work history, there is a real and substantial likelihood that he would have worked part-time from January, 2018 to present.
- [336] In reaching this conclusion, I have considered the evidence of the Defendants' Vocational Rehabilitation expert, Diamantis Zervas. Mr. Zervas prepared a detailed report. One of the premises of his report is that Mr. West would have less than a 5% chance of returning to the workforce after an absence of two years. This is based on research. It is accepted that as of the date of the accident, Mr. West had been out of the workforce for two years. However, he had returned to school to retrain. I find Mr. Zervas's 5% statistic inapplicable, because it is undermined by the fact that after two years of not working or going to school, Mr. West returned to school and successfully completed courses.
- [337] I am therefore more optimistic about his pre-accident and post-accident working potential than the Defendants.
- [338] Mr. Zervas examined Mr. West's transferrable skills and found that if he were able to work, he could work in many capacities, pending medical clearance. These included assembler, packager, process control operator, cashier and counter attendant. The wage range of these jobs was from \$16.55 to \$33.65 per hour. That there are other jobs in which Mr. West could work accords with Mr. West's evidence. He testified that he had worked in kitchens. This was not known to Mr. Zervas as it is not set out in documentary evidence although the Ontario Works file contains a resume for Mr. West.
- [339] I find that once finishing school, Mr. West would have worked. I find that he would have worked from January 1, 2018 to January, 2025 after obtaining a high school diploma. I accept that he would have continued to work in low-skilled jobs off and on.
- [340] The Plaintiff and the Defendants each tendered economic loss reports. The Plaintiff's scenarios assume that Mr. West would work full-time which was not realistic in my view.

The range is from \$354,778 where Mr. West would have earned 2021 Statistics Canada average full-year full-time employment for a male with a high school or equivalent to \$222,768 which assumes that Mr. West would have completed college and started working in September 2018. The Defendants' expert presented three scenarios. The Defendants' scenarios are nil to \$88,000.

- [341] I find that the Defendants' opinion is too low as it relies on 2016 census data and does not adjust for age. Mr. Durrani's calculations do not account for Mr. West's inability to sustain full-time work. I find that to reach the proper result, I award one-half of \$354,778. I find that the Plaintiff has demonstrated that he would have been able to return to work of some kind, at least on a part-time basis with a high school equivalency.

(iii) Conclusion on Past Income Losses

- [342] I find that Mr. West is entitled to past income loss from January 1, 2021 to January 2025, in the amount of \$177,389.

(c) Future Economic Loss

- [343] The Plaintiff seeks compensation for future economic loss.

(i) Test

- [344] To be compensated for future economic loss, the Plaintiff must establish that there is a real and substantial risk of that loss. See *Beldycki Estate v. Jaipargas*, 2012 ONCA 537, at para. 75.

(ii) Evidence

- [345] I heard from the Plaintiff's expert, Saqib Durrani, the Defendant's expert, Hannah McCannell and Mr. Zervas testified.
- [346] I have found above that I do not accept that the Plaintiff has demonstrated that there is a real and substantial possibility that Mr. West would have gone on to be a Radiation Safety Officer. I have accepted that he would work in one of the jobs identified by Mr. Zervas.
- [347] I must now determine the loss. I have some difficulties with Ms. McCannell's evidence. Ms. McCannell's evidence relied on Mr. Zervas' evidence arguing that it shows that Mr. West retains a residual earning capacity. That is not how I understood his evidence. Mr. Zervas' evidence was that if he was to be employable, which he deferred to a medical person, that he was employable in a number of capacities (final assembler – packager and other jobs). I find the evidence does not support that the Plaintiff can work and the lay evidence does not support that either.
- [348] I also note that Ms. McCannell relied on 2016 census data. She did not assess for Mr. West's age and in my view, her estimates result in calculations which are too low.

- [349] Ms. McCannell was critical of Mr. Durrani's calculations because there was no support for the fact that Mr. West worked in a full-time capacity. She found that there was no evidentiary support that Mr. West would have retained a job in radiation safety based on her review of the documentation provided relating to her pre-accident employment and medical history. However, I cannot accept that this aspect of her opinion properly flows from her expertise, despite that I have ultimately come to the same conclusion based on the evidence that I heard at trial.
- [350] Given that I find that Mr. West would likely continue as he did when he was working, in lower skilled jobs, with periods of either not working or being laid off, I accept Mr. Durrani's averaging of his historical earnings from 2008 to 2014 which is \$35,933 in 2025 dollars. I adopt this approach set out in Table 14 of Mr. Durrani's report and calculate the present value of Mr. West's future income loss at \$724,844.

(iii) *Conclusion on Future Economic Loss*

- [351] I find that the Plaintiff has demonstrated on the real and substantial risk standard a future economic loss of \$724,844.

(d) Future Cost of Care

- [352] Future cost of care is assessed on a real and substantial likelihood: *Schrump et al. v. Koot et al.*, 1977 CanLII 1332 (ON CA), that is whether there is a real and substantial risk that Mr. West will require the care.
- [353] It is based on what is reasonably necessary on the medical evidence. See *Fraser v. Persaud*, 2023 ONSC 1449, at para. 628. The award must be reasonable to both the Plaintiff and the Defendant.
- [354] I have rejected that Mr. West suffered only soft tissue injuries.
- [355] I heard evidence from Marla Tennen who I qualified as an expert in future care costs and life planning. I also heard from the Defence expert Dr. Andrew Holland who critiqued Ms. Tennen's report because many of the recommendations were not supported by the evidence.
- [356] Ms. Tennen recommended case management, a rehabilitation support worker, housekeeping, psychological counselling with cognitive behavioural therapy, social work interventions, THC oil, various assistive devices, and attendant care.
- [357] Dr. Holland testified that there was no clear evidence for Ms. Tennen's recommendations. He stated that there was no evidence that Mr. West had received the support of a rehabilitation support worker, and that no regulated health professional had recommended social work support. He found the hours claimed for psychological support excessive, but he agreed that a psychological assessment was supported. He stated that there is no evidence that Mr. West requires daily attendant care.

- [358] I find that there is no support for THC oil and the assistive devices Ms. Tennen recommends. Similarly, I find that Mr. West has rejected treatments delivered by way of injections in the past and I do not find it realistic that he will seek treatment by injection in the future.
- [359] I agree that some of what Ms. Tennen recommends is excessive, but I do not reject her evidence entirely.
- [360] I find that Mr. West did not require psychological services pre-accident but that he now requires it. Prior to the accident, Mr. West managed with support from his family doctor, and occasional attendances at hospital when in crisis. Post-accident, he relied on Dr. Thomas, and he attended with difficulty. I find that if he had had support with attendance at appointments, he would have likely had fewer absences.
- [361] This is why I find support for a rehabilitation support worker. He identified structural barriers with getting to care and had difficulty keeping appointments. This is why I find that one is reasonably necessary. This is also supported by Mr. West's evidence. Mr. West did not access services upon the settlement of his accident benefits claim. However, his explanation, which I accept was that he needed money to live and to pay off debts. I accept that Mr. West lived in poverty post-accident having lost his social supports and his ability to earn income.
- [362] I also note that Mr. West had identified that he receives some support from his brain injury worker. He has identified him as being of benefit to him. I find that there should be an award for funding for a rehabilitation support worker, for psychological services, housekeeping, and for case management. I do not find support for attendant care or social work.
- [363] Dr. Holland's recommendations for assessments do not adequately provide for Mr. West's reasonably necessary care based on a real and substantial risk of being required.
- [364] In terms of life expectancy, I accept, from Ms. Tennen's evidence, that the average life expectancy for a 36-year-old man is an additional 45.4 years.
- [365] I award damages as follows.:
- a. One neuropsychological assessment at \$6,000;
 - b. A one-time occupational therapy assessment \$600;
 - c. Rehabilitation support worker for six hours per week, 48 weeks per year for years 1 to 3 at a rate of \$65, the present value of which is \$55,093. Then, for three hours per week at 48 weeks per year for life. The annual cost of which is \$9,360 and the present value multiplier being 26.562 for a total of \$248,620;
 - d. Psychological services and cognitive behavioural therapy as recommended in report (48 sessions per year in years 1 to 2, two sessions per year in years 3 to 5 and

years 6 to age 65, once monthly at \$220 per session) for a total present value of \$20,856 + 15,143 + 14,806 totalling \$50,805;

- e. Housekeeping services in the amount of seven hours per week at \$26.00 per hour to age 75 at 50 weeks per year which is \$9,100. Using a multiplier of 25.617, this totals \$233,115;
- f. Case management for two years with a present value of \$11,850. I find that there is no evidence that case management services are required for life.

[366] The total award for the cost of future care is \$396,083.

[367] The final costs of future care are reduced by \$110,000 which Mr. West received through the settlement of his accident benefits claim resulting in an award of \$286,083.

(e) Out of Pocket Expenses

[368] The Plaintiff has proven out of pocket expenses in the amount of \$23,924.84.

E. Threshold Governing Principles

[369] The general rule is that no person is entitled to general damages or health care expenses arising from the use of a motor vehicle unless the person has sustained a serious disfigurement or a permanent and serious impairment of an important physical, mental or psychological function. This is a threshold question. If the injuries do not meet that statutory exception, the *Insurance Act* prohibits recovery.

[370] The *Insurance Act* also contains a statutory deductible which operates to reduce awards of damages if they do not meet regulated amount.

[371] As set out in *Meyer et al. v. Bright et al.* (1993), 1993 CanLII 3389 (ON CA), 15 O.R. (3d) 129 (C.A.) (“*Meyer*”), the threshold question is determined by examining three questions:

- a. Has the injured person sustained permanent impairment of a physical, mental or psychological function?
- b. If yes, is the function which is permanently impaired important?
- c. If yes, is the impairment of the important function serious?

[372] These are defined by regulation. The regulation also mandates the evidence required to meet the threshold. Section 4.2 of O.Reg 461/96 under the *Insurance Act*, provides:

(1) A person suffers from permanent serious impairment of an important physical, mental or psychological function if all of the following criteria are met:

1. The impairment must,
 - i. substantially interfere with the person's ability to continue his or her regular or usual employment, despite reasonable efforts to accommodate the person's impairment and the person's reasonable efforts to use the accommodation to allow the person to continue employment,
 - ii. substantially interfere with the person's ability to continue training for a career in a field in which the person was being trained before the incident, despite reasonable efforts to accommodate the person's impairment and the person's reasonable efforts to use the accommodation to allow the person to continue his or her career training, or
 - iii. substantially interfere with most of the usual activities of daily living, considering the person's age.
2. For the function that is impaired to be an important function of the impaired person, the function must,
 - i. be necessary to perform the activities that are essential tasks of the person's regular or usual employment, taking into account reasonable efforts to accommodate the person's impairment and the person's reasonable efforts to use the accommodation to allow the person to continue employment,
 - ii. be necessary to perform the activities that are essential tasks of the person's training for a career in a field in which the person was being trained before the incident, taking into account reasonable efforts to accommodate the person's impairment and the person's reasonable efforts to use the accommodation to allow the person to continue his or her career training,
 - iii. be necessary for the person to provide for his or her own care or well-being, or
 - iv. be important to the usual activities of daily living, considering the person's age.
3. For the impairment to be permanent, the impairment must,
 - i. have been continuous since the incident and must, based on medical evidence and subject to the person reasonably participating

in the recommended treatment of the impairment, be expected not to substantially improve,

ii. continue to meet the criteria in paragraph 1, and

iii. be of a nature that is expected to continue without substantial improvement when sustained by persons in similar circumstances.

(2) This section applies with respect to any incident that occurs on or after October 1, 2003.

[373] Section 4.3 of the Regulation addresses the necessary evidence:

(1) A person shall, in addition to any other evidence, adduce the evidence set out in this section to support the person's claim that he or she has sustained permanent serious impairment of an important physical, mental or psychological function for the purposes of section 267.5 of the Act..

(2) The person shall adduce evidence of one or more physicians, in accordance with this section, that explains,

(a) the nature of the impairment;

(b) the permanence of the impairment;

(c) the specific function that is impaired; and

(d) the importance of the specific function to the person.

(3) The evidence of the physician,

(a) shall be adduced by a physician who is trained for and experienced in the assessment or treatment of the type of impairment that is alleged; and

(b) shall be based on medical evidence, in accordance with generally accepted guidelines or standards of the practice of medicine.

(4) The evidence of the physician shall include a conclusion that the impairment is directly or indirectly sustained as the result of the use or operation of an automobile.

(5) In addition to the evidence of the physician, the person shall adduce evidence that corroborates the change in the function that is alleged to be a permanent serious impairment of an important physical, mental or psychological function.

(6) This section applies with respect to any incident that occurs on or after October 1, 2003.

F. Do the Injuries Meet the Threshold?

- [374] Above I have commented on the injuries suffered by Mr. West, and I find that they are impairments within the meaning of the statute.
- [375] I find that Dr. Ko's report is sufficient evidence under s. 4.3 of the Regulation. I find that the Plaintiff has adduced evidence that corroborates Dr. Ko's evidence relating to the change in Mr. West's function. The Plaintiff has done that through the Plaintiff's evidence, through the evidence of his mother and Ms. Robinson as well as the evidence of Dr. Randhawa, Dr. Edwards, Dr. Miners and Dr. Thomas.
- [376] Ms. Robinson and Mr. West described the challenges that he faces with basic functions. Ms. Robinson described that he could not carry all the groceries to his apartment on his own. He could not shop for himself. He has difficulty with a number of chores.
- [377] I find also that Dr. Appleton's evidence supports other impairment.
- [378] I am convinced on evidence that meets the requirements of the Regulation, that the functions impaired are important ones.
- [379] The evidence also establishes that the impairments are serious.
- [380] These functions are necessary for Mr. West to provide for his own care or well-being as well as being important to his usual activities of daily living, considering his age. That he has been unable to do them is evident in his post-accident decline. Both the lay and the medical and expert evidence support this conclusion.
- [381] As for permanency, permanent means lasting indefinitely into the future as opposed to for a limited time with a definite end. See *Kayhan v. Greve*, [2011] OJ No. 465, at para. 26. Here, the evidence establishes on a balance of probabilities that these conditions will not resolve, and I am satisfied that the Plaintiff has established permanence.
- [382] I therefore find that the Plaintiff has met the threshold. Therefore, I dismiss the Defendants' threshold motion.

V. Conclusion

- [383] For the reasons given above, I dismiss the Defendants' threshold motion. The action is allowed. I award damages to the Plaintiff in the amount of \$1,388,316 representing:
- a. General damages for pain, suffering and the loss of enjoyment of life in the amount of \$200,000.

- b. Damages for past income lost in the amount of \$177,389.
- c. Damages for future income loss in the amount of \$724,844.
- d. Damages for the cost of future care, less statutory accident benefits already paid out, \$286,083.

[384] Judgment shall issue in accordance with this decision. If there are any arithmetic errors that need to be addressed, I may be contacted through my Judicial Assistant.

[385] The parties are urged to agree on costs. If they cannot do so by January 15, 2026, they shall so advise me through my Judicial Assistant. I will deal with costs by way of written submissions. If the parties cannot agree on costs, they should attempt to agree on a timetable for the exchange of their written submissions. The parties should contemplate that they will be restricted to a five-page double-spaced submission exclusive of Bill of Costs and offers. I do not wish to impose a timetable without counsel's input given the holidays and the upcoming Central East Regional Trial sittings.

[386] I thank the counsel for their assistance throughout and for their civility during the trial. I also thank them and the parties for their patience in waiting for this decision.

Justice S.E. Fraser

Released: December 17, 2025

CITATION: West v. Hanna, 2025 ONSC 7088

ONTARIO

SUPERIOR COURT OF JUSTICE

BETWEEN:

DOUGLAS WEST

Plaintiff

– and –

ALEX J. HANNA and TRANSFORCE
ADMINISTRATION INC. aka TRANSFORCE
ADMINISTRATION INC./ADMINISTRATION
TRANSFORCE INC., CANPAR TRANSPORT
L.P./TRANSPORT CANPAR, S.E.C. and TF1
TRANSPORT 2 L.P./TRANSPORT TF1 2, S.E.C.

Defendants

REASONS FOR JUDGMENT

Justice S.E. Fraser

Released: December 17, 2025